

	Questions	Rational e	Response
1	Good morning! how are you? Thank you again for participating in this interview. Again, we <i>do</i> believe your experience and perspective would bring much to the project. Before we begin, is it alright if we record this interview? It will just be for transcription for note-taking, but I will not be keeping the video.	Rapport	Hi! I'm glad I could help.
2	To start, I just want to get a better sense of your practice, [repeat here any information that is publicly available: Specific modality, client profile, country of origin] Is that correct? IF unknown modality: What kind of couple's therapy would you say you primarily provide? (CBCT, EFT, Family Systems) IF unknown client profile: Are there specific types of couples that you provide your services to? IF uncertain country of practice: And your provide services to couples in [country], correct?	Information about clinical practice (+ clinical utility later on)	I have been a couples therapist for over [25-30] years. I started my training with Systemic Therapy, then with Cognitive Behavioral Therapy. In [Year], I discovered Contextual Behavioral Therapies (ACT) and I was curious about what couples therapy would be like in this approach. That's when I met Integrative Behavioral Couple Therapy (IBCT) and I was enchanted. I started my training with the author [Name], I am the first [Nationality] with complete training in IBCT (training, workshops, supervision and supervision with videos of attendance of various therapeutic processes). Even more, I did my doctorate in this approach evaluating the therapeutic processes of this couple therapy. Among several instruments and forms of evaluation (the IBCT has a specific protocol), I used a variation of the PQS (PSYCHOTHERAPY PROCESS Q-SET) for couples, the CTQS (COUPLE PSYCHOTHERAPY Q_SET). I work with couples in [Country 1] and, in the online

			modality, in several countries in [Country 2].
3	When clients come to you, what is usually the presenting issue or the reason that they seek your service?	Information about clinical practice	Most couples look for difficulties in communication, understanding what is happening with the other person and a desire to change the other. Many are at a very high level of wear and the fights are more intense.
4	1) Do you do a formal assessment? Why or why not? 2) How do you do the assessment and how long does it take? 3) Probe: Are there relationship scales that you use? When do you use these scales? How does this fit into the assessment? 4) Probe: How informative do you find these scales to be? 5) Probe: Do you have any challenges when using these scales? 6) Probe: If its alright with you, can I ask how much these scales cost you?	Information about clinical practice	1) Yes, I do the evaluation and formulation of the case from the IBCT protocol (which is great!) I don't see how a therapist works without doing a good evaluation of the case before starting the therapeutic process. This evaluation lasts 4 meetings: the first with the couple, the following two are individual (each answering questionnaires that help in the evaluation) and the fourth meeting is with the couple, for the return of the evaluation. This is also great because the couple knows and owns the formulation of the case. 2) I follow the IBCT protocol, it takes 4 meetings (1 month) 3) Yes, in the protocol there are specific scales of satisfaction, personality differences, problem areas, as well

			as feedback to the therapist and assessment of the therapeutic alliance. They are used at the beginning, in the evaluation and formulation phase of the case. 4) Very informative, they are validated and help both the therapist and the couple 5) In [Nationality] culture, people don't usually answer evaluation questionnaires, this was a challenge and I had to adapt to answer with me, in the session. In the inline mode, I send a reminder about the importance of responding and sending it to me before the individual session 6) They are free, available on [Name]. I did the translation into [Language 1] and [Language 2] and also makes it available to therapists
5	Across the years, different therapists and different researchers have characterized the construct of relationship quality quite differently from each other. I wanted to get your view about it, for you, what is relationship quality?	Therapist conceptualization of relationship quality	In a very open way: feeling of well being with the partner, acceptance of things that will not be changed, and communication and problem-solving skills. This is because life imposes many adversities and what represents mental health is the ability to adapt.
6	1) For you, what differentiates couples with high relationship quality and those with low relationship quality?		1) High quality: couples who manage to move between acceptance (of what cannot be changed in the other.

	2) Probe: In your professional experience, what are the most important things in a (high-quality) relationship? Why do you say so?		Obviously, I'm not talking about violence or abusive situations here, but personality differences, ways of dealing with stress, desire for intimacy) and change (what really needs and can be changed); casasi who have common dreams and move towards what is important for their lives. 2) Each one is aware of their own internal world, of their emotional wounds and triggers, so that they can choose their behavior. It is also important for the other to know your partner's story to have more understanding and empathy.
7	Does how you conceptualize relationships often differ from how your clients conceptualize relationships? In what way? Does this matter for therapy?		In general, yes. In the beginning they want to change the other and, when the formulation is presented, it is already a form of intervention, of psychoeducation about the existence of a relationship in which both contribute.
8	The relationship is a complex system, it involves love, sex, idenity, relationship maintenance, conflicts, responsiveness, satisfaction etc. During therapy, how do you decide which one to focus on or which one in the complex system to target?	Therapist conceptualization of relationship quality	At the IBCT, when the case formulation is presented, the couple participates and chooses the main problems to be worked on (sex, parenting, finances, time together, etc). From then on, in the active phase of therapy, the topic is worked on, including a weekly questionnaire that helps organize the session's agenda. But, the big focus is not the theme in particular, but the

			interaction around the theme, the emotional wounds touched and the way they deal with it, usually in a dysfunctional way.
9	1) How do you determine when to end the couple's therapy? [If not answered] Probe: What are the usual treatment goals? 2) How long are the sessions?	Information about clinical practice	1) Discharge is a process in which we observe, together, how the couple is dealing with life issues: if they are able to talk about an issue, if this is not a problem, if they know their emotional sensitivities and have communication and resolution skills. of problems. If they can handle it, they are ready to move on with life. The important thing is to be aware that therapy is not for solving problems, but for them to have the skills to deal with life's issues. 2) The IBICT has several investigations that prove its effectiveness. The average of meetings is 24 sessions. The duration of each session is around 50 minutes
1 1	1) We are currently developing a relationship quality scale that hopefully better represents the relationship as a complex system, that would help therapists in treatment planning and evaluation, how interested would you be in this scale? Personally, What would you be looking for in this newly developed relationship scale? How can we make sure that this scale would be useful in your practice?	Clinical utility	1) I think it's very important

	2) If this scale were to exist, would there be enough time in your sessions to integrate it either in assessment and/or evaluation? Would this work better for you online or offline? Probe: We are planning to carefully develop and validate this scale over the course of two years, after which we will be selling it for [redacted] euros (lifetime license, unlimited copies) -- until we recoup the development costs after which it will be free. Would this cost be feasible for you? Why or why not? 3) Probe (if they believe it to be too expensive): If every license brought donates two licenses to therapists in low-and-middle income countries, would this be more appealing to you? .		2) I don't know how it would be, because I already use the IBCT scales. But I would like to meet I think the dissemination of knowledge is very important, Contextual therapies have the philosophy of sharing everything, in the desire to help more people
1 2	Would you be interested to collaborate on the development of the scale?	[Send updated proposal]	To answer I need to know how it would be
1 3	Thank you so much! Now before we end, I want to talk to you about how we will handle the data. I wanted to know which data sharing option you are most comfortable with One option is transcript portions can be shared, none of which can be identified to you Another option is that the whole transcript can be shared removed of sensitive and overly identifying details, so it will be anonymized. Although in this option, there is very minimal chance that someone who knows you well professionally can piece together the details in the transcript (country, therapy practice etc.) and narrow down who among their colleagues they believe it is. Although again, no sensitive information will be shared If you're not comfortable with any of these options, let me know and we can discuss other data sharing options.		You can share what you think is important. Hope this helps. I hope you manage to do an excellent job in your investigation.

